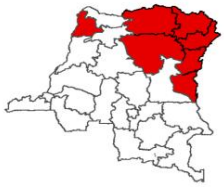
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Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
PRESIDENTIAL EBOLA TASK FORCE 17
MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°161/MVE-BDBV/22/10/2026

Reporting date: October 22, 2026 Publication date: October 23, 2026

HIGHLIGHTS (last 24 hours)

In total, 51 new confirmed cases and 30 deaths (12 community deaths and 18 intra-ETC deaths) of Ebola Virus Disease (EVD) cases have been reported in the provinces of Ituri (27), North Kivu (20) and Haut-Uélé (4). A total of 545 new samples were analyzed; 51 of these samples tested positive for the Bundibugyo virus, representing a positivity rate of 9.4%.
No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CAS	ACCUMULATIVE CONFIRMED DEATHS	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
9 816	4757	48,5%	1130	2 635	89,1 %

Affected provinces and affected health zones

8 Provinces affected

Uele, Ituri, North Kivu, South Kivu, Tshopo, Lower Uele, South Ubangi, and Nord Ubangi

66 Health Zones affected out of 178 (37.1%)

Summary of key points from the last 24 hours

As **of October 22, 2026, 51** new confirmed cases of Ebola virus disease (EVD) were reported in three provinces: **Ituri (27 cases), North Kivu (20 cases) ,** and **Haut-Uélé (4 cases).** Among these cases, **12 community deaths** were reported (6 in North Kivu, 5 in Ituri, and 1 in Haut-Uélé). In addition, **18 confirmed deaths** were recorded in Ebola Treatment Centers (ETCs) (13 in Ituri and 5 in North Kivu), bringing **the total number of confirmed deaths** reported during this period to 30.

o In addition, **17 patients (9 in North Kivu and 8 in Ituri)** were declared cured after obtaining two negative results in the control test for Ebola virus disease.

As of October 22, 2026, the DRC has accumulated **9,816 confirmed cases, including 4,757 deaths,** representing a case fatality rate of **48.5** %. Ituri province remains the epicenter, accounting for **72.4%** of reported confirmed cases and **52.9%** new confirmed cases.

o No new health zones have been affected in the last 24 hours. Thus, 66 out of 178 health zones (37.1%), spread across eight provinces, have been affected since the start of the epidemic.

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Table 1. Distribution of confirmed cases and deaths by affected province, as of October 22, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	27	7107	3 316	46,7%	28/36 (77,8 %)
North Kivu	20	2183	1 221	55,9%	16/34 (47,1 %)
Haut-Uélé	4	429	186	43,4%	7/13 (53,8 %)
Boss	0	76	27	35,5%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	0	13	4	30,8%	4/11 (36,4%)
Sud Ubangi	0	2	1	50,0%	1/16 (6,3%)
Nord Ubangi	0	3	1	33,3%	1/11(9,1%)
Total	**51**	**9 816**	**4 757**	**48,5%**	**63/167 (37,7 %)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 22, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**7107**	**3316**	**46,7%**	**27**	**5**	**13**	**18**
Give	11	1	9,1%				0
Understand	20	6	30,0%	3	0	0	0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	216	48	22,2%				0
have	2	2	100,0%				0
Bunia	1927	651	33,8%	4	2	0	2
Ladies	41	10	24,4%				0
Drodro	15	7	46,7%				0
Fireworks	84	31	36,9%				0
Gethy	11	7	63,6%				0
Flounder	2	0	0,0%				0
Kilo	38	12	31,6%				0
Team	192	118	61,5%	1	1	0	1
Liter	317	158	49,8%	2	0	0	0
Logo	14	9	64,3%				0
Fight	36	9	25,0%				0
distributed	5	1	20,0%				0
Washing	78	34	43,6%	1	0	1	1
Darkness	387	210	54,3%	7	2	7	9
Mangala	384	168	43,8%	2	0	2	2

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Mongbwalu	717	346	48,3%				0
Nia -Nia	337	197	58,5%			1	1
Nizi	837	306	36,6%	1	0	0	0
Legumes	134	41	30,6%				0
Jungle	21	5	23,8%	2	0	0	0
Rwampara	1179	414	35,1%	4	0	2	2
Tchomia	82	25	30,5%				0
A valves	THAT	493	THAT				0
North Kivu	**2183**	**1221**	**55,9%**	**20**	**6**	**5**	**11**
Me	560	379	67,7%	8	4	3	7
Farm	24	8	33,3%				0
Butembo	323	190	58,8%				0
Ten	1	0	0,0%				0
Kalunguta	87	32	36,8%	3	1	0	1
Cut	653	399	61,1%	2	0	1	1
Source	3	2	66,7%				0
Kyondo	44	21	47,7%				0
Lubero	37	14	37,8%				0
Be careful.	88	31	35,2%	2	0	0	0
Alimbong	5	3	60,0%				0
Manguredjipa	16	5	31,3%				0
Masereka	39	13	33,3%				0
The museums	154	72	46,8%				0
Mutwanga	35	10	28,6%	2	0	1	1
Oicha	59	26	44,1%	3	1	0	1
Vuh	55	16	29,1%				0
Upper Uele	**429**	**186**	**43,4%**	**4**	**1**	**0**	**1**
Government Mangbetu	45	19	42,2%				0
Dumb	1	1	100,0%				0
Gombari	3	1	33,3%				0
Mathematics	108	48	44,4%				0
Lining	107	61	57,0%				0
Beans	6	3	50,0%				0
Ordinary	159	53	33,3%	4	1	0	1
Boss	**76**	**27**	**35,5%**	0	0	0	0
Bafwasende	18	3	16,7%				0
Kabondo	10	2	20,0%				0
Lubunga	1	0	0,0%				0
Maquis - Kisangani	33	14	42,4%				0
Mangobo	10	7	70,0%				0
Boss	3	0	0,0%				0

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Wanie-Arugula	1	1	100,0%				0
South Kivu	**3**	**1**	**33,3%**	0	0	0	0
Miti-Murhesa	3	1	33,3%				0
Bas Uélé	**13**	**4**	**30,8%**	0	0	0	0
Blind	1	1	100,0%				0
Viadana	6	1	16,7%				0
Claim	1	0	0,0%				0
Walk	5	2	40,0%				0
Sud Ubangi	**2**	**1**	**50,0%**	0	0	0	0
Hair	2	1	50,0%				0
Nord Ubangi	**3**	**1**	**33,3%**	0	0	0	0
You are logged in.	3	1	33,3%				
Total	**9816**	**4757**	**48,5%**	**51**	**12**	**18**	**30**

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA : Not Applicable.

Table 3. Status of alerts notified by province for October 22, 2026

DPS	Alerts Received		Total alerts	Verified alerts and validated (suspected case)		Verified alerts and invalidated (non-suspected case)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	134	4	138	20	4	114	0	16	9
Bas Uélé	189	3	192	20	3	169	0	23	0
Ituri	1 272	56	1 328	145	53	1 127	3	191	126
North Kivu	1 297	51	1 348	137	51	1 160	0	162	124
Sud Ubangi	ND	ND	ND	ND	ND	ND	ND	ND	ND
Nord Ubangi	ND	ND	ND	ND	ND	ND	ND	ND	ND
Boss	97	1	98	17	1	80	0	18	2
Grand Total	**2 989**	**115 3 104**		**339**	**112**	**2 650**	**3**	**410**	**261**

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Figure 1. Evolution of cases by date of symptom onset over the last 21 days

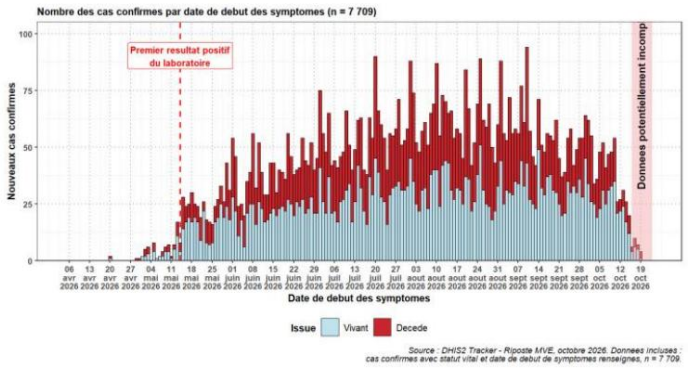
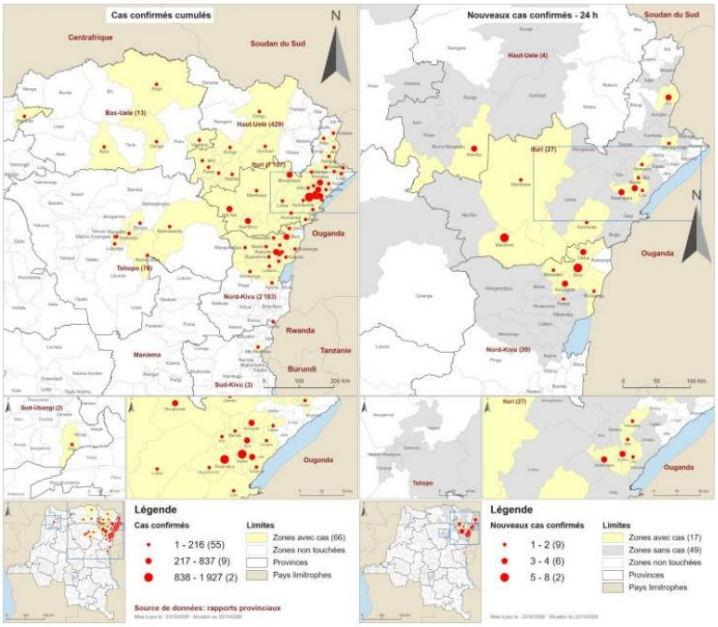


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

ÿ Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

ÿ As of October 22, 2026, 3,104 alerts had been reported in the five provinces that submitted their data, including 2,989 alerts of living persons and 115 of deaths. Of these alerts, 451 (14.5%) were validated as suspected cases and 2,653 (85.5%) were invalidated. Of the 451 validated suspected cases, 410 (90.9%) were investigated, of which 261 (63.7%) were transferred to COVID-19 treatment centers; 41 suspected cases remain uninvestigated (26 in North Kivu, 8 in Haut-Uélé, and 7 in Ituri).

ÿ As of October 22, 2026, 28,067 contacts were under follow-up in the five provinces that submitted their data; 25,005 were seen in the last 24 hours, representing a follow-up rate of 89.1%, up from the previous day (84.8%) but below the required threshold of 95.0% (Table 4). By province, the follow-up rate was 86.2% in Ituri (10,999/12,758), 92.5% in North Kivu (10,360/11,203), 89.8% in Haut-Uélé (1,788/1,992), 81.4% in Tshopo (1,099/1,350) and 99.3% in Bas-Uélé (759/764).

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

ÿ On October 22, 2026, sixteen (16) alerts were reported: 4 in Ituri, 3 in North Kivu, 8 in Bas-Uélé, and 1 in Tshopo. In Ituri, all 4 active alerts were validated and transferred (Tchaï to the Nyankunde Ebola Treatment Center, Bunia Airport and Mudzi Bala to the Bunia Ebola Treatment Center, and Rubingo to the Aveba ESS). In North Kivu, 2 out of 3 alerts were validated (Maboya, referred to the Tinge Hospital, and Kangote, whose traveler refused referral), and the body of a one-year-old child, confirmed positive for Ebola, was intercepted at the Foner Kasindi Point of Control in a public transport vehicle coming from Mutwanga; the body was sent to the morgue for EDS. In Bas-Uélé, all 8 alerts were invalidated after verification. In Tshopo, 1 live alert was notified to the private port Office de route (Makiso-Kisangani) and was not validated.

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Table 4. Status of key PoE/PoC indicators by province, as of October 22, 2026

Indicators	Ituri	North Kivu	South Kivu	Boss	Haut-Uélé	Bas-Uélé	Global
Report completeness	95% (59/62)	90% (19/21)	56% (5/9)	100% (18/18)	90% (9/10)	100% (20/20)	93% (130/140)
Strategic PoC/PoE	[1]	0	0	0	0	0	[1]
Number of past travelers	148 084	99 012	6 931	20 483	19 526	11 780	305 816
Proportion of travelers screened	97%	98%	98%	99%	99%	98%	98%
% of travelers who washed their hands	97%	98%	98%	99%	99%	99%	98%
% of the travelers sensitized	99%	98%	100%	100%	100%	93%	99%
Number of alerts notified	04	03	0	01	0	08	16
Number of validated live alerts	04	02	0	0	0	0	6
Number of lifeless bodies intercepted	00	01	0	0	0	0	[1]
% of suspected cases transferred to CT/CTE	100% (4/4)	50%	0%	0%	0%	0%	83% (5/6)
% of lifeless bodies swabed/secured	0%	100%	0%	0%	0%	0%	100% (1/1)
% of suspected cases investigated within 24 h	100% (4/4)	67%	0%	0%	0%	0%	86% (6/7)

2.3. Laboratory

2.3.1. Main actions

- **Ituri: 27 new positive results** (22 living and 5 dead) out of 232 new samples analyzed (positivity: 11.6%).

- North Kivu: 20 new positive results (14 living and 6 dead) out of 278 new samples analyzed Beni and Butembo (positivity: 7.2%).

- **Haut-Uélé: 4 new positive results** (3 living and 1 death) out of 21 new samples analyzed (positivity: 19.0%).

- **Tshopo: 12 samples received and analyzed** (11 living and 1 deceased); the results were negative

- **Bas Uélé: 2 samples received and analyzed** (2 live) the results are revealed negative.

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri :** 25 out of 41 rings were opened (61.0%), 32 out of 43 households were decontaminated (74.4%), and 5 out of 5 health facilities were decontaminated (100.0%). Of 76 EDS alerts (including 13 postponements), 47 EDSs were carried out (61.8%).

- **North Kivu :** 16 out of 28 rings were opened (57.1%), 19 out of 23 health centers were decontaminated (82.6%), and 6 out of 19 households (31.6%). 14 infection prevention and control (IPC)/water, sanitation, and hygiene (WASH) assessments were conducted in 14 health centers (average score of 50.5%), 43 schools were equipped with handwashing stations in Katwa, 10 school brigades were briefed in Mabalako, and 14 public places were decontaminated. Of 61 death and health alerts received, 61 death and health assessments were carried out, and 59 bodies were swabed.

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ÿ **Haut-Uélé :** 1 out of 1 ring road was opened, 3 out of 3 households and 2 out of 2 health centers were decontaminated in Pawa, 28 service providers were briefed in 4 health zones and 3 health centers were assessed (Badamoni Health Center 28.8%, Bakapele Health Center 4.8% and Ibambi State Health Center 53%). Out of 6 health alerts, 3 health assessments were carried out (2 in Wamba and 1 in Pawa).

ÿ **Tshopo :** 5 rings were opened (Tshopo 2 and Bafwasende 3), 6 ESS followed in 3 ZS and 117 providers briefed; the CM GESOM (case n°75), the CSR Bafwanduo, the CM Tout va bien, the CS Bafwazana and the CM Saint François d'Assise (case n°71) were decontaminated and 18 PPL assessed, all at high risk; the HGR Mangobo and the CS Maman Mwilu were reassessed (74.2% and 83.9%).

ÿ **Bas-Uélé :** 8 out of 8 ESS were followed in 3 ZS (Buta, Ganga and Viadana), 52 out of 52 providers and 11 out of 11 hygienists were briefed in 5 ESS and 3 scorecard assessments were carried out (HGR Buta 72%, HGR Viadana 50% and CSR Neru 43.5%); 1 body was swabé in Agri-Uélé (Ganga).

2.4.2. Challenges.

In Tshopo, household resistance prevented the investigation of case #54, and only 6 out of 106 health facilities (5.7%) achieved an IPC score exceeding 80%. In Bas-Uélé, two swab failures were recorded in Ganga, no DHS was conducted, 16 out of 20 priority health facilities lacked functional triage, 17 out of 20 lacked water, and only 130 out of 632 service providers (20.6%) were trained in IPC-WASH. In Haut-Uélé, two DHS could not be conducted due to community resistance (Boma Mangbetu and Legu health facilities), and only 4 out of 7 health zones reported IPC in the health facilities.

2.5. Holistic care
2.5.1. Main actions

ÿ **In Ituri,** 102 new admissions were recorded compared to 102 discharges, including 8 recoveries and 2 discharges without medical advice. Thus, 499 patients are hospitalized, representing an occupancy rate of 49.2%, including 64 critically ill patients; 471 out of 491 patients (95.9%) received hot meals, and a new CT (Center for Treatment) is operational in Bafwambaya (Nia-Nia).

In North **Kivu,** 57 new admissions were recorded, compared to 69 discharges, including 9 recoveries, 2 discharges without medical advice, and 6 deaths (5 confirmed). A total of 477 patients are currently receiving care (178 confirmed and 299 suspected), including 61 children under 5 years old and 8 critically ill patients, 138 of whom are outside of Ebola Treatment Centers (ETCs), for an overall occupancy rate of 82.7% (339/410 ETCs). 27 ambulance trips were successfully completed (11 out of 14 ambulances used).

ÿ **In Haut-Uélé,** 14 new admissions (1 confirmed and 13 suspected) were recorded against 15 discharges (13 non-cases and 2 transfers); 35 patients are hospitalized (10 confirmed and 25 suspected, including 1 confirmed critical) for 184 beds, representing an occupancy rate of 19.0%.

ÿ **In Tshopo,** 4 new admissions of suspected cases were recorded with no discharges; 13 patients are hospitalized (4 confirmed and 9 suspected, including 1 severely ill) for 36 beds, representing an occupancy rate of 36.1%.

ÿ **In Bas-Uélé,** 20 new admissions of suspected cases were recorded and isolated in the ESS against 2 discharges (non-cases in Koteli), bringing the number of patients in isolation to 106; the 3 beds for confirmed cases are occupied (100.0%).

2.5.2. Challenges

ÿ **In Ituri,** blood remains insufficient, 20 patients from Mandima (CT Mandima and CT Makeke) have not received nutrition and the capacity to receive patients remains low in Aru, Mambasa, Kilo, Lolwa, Mandima and Biakato.

ÿ **In North Kivu,** the beds of suspected cases are occupied at 84.9% (163/192) and those of confirmed cases at 80.7% (176/218), 138 out of 477 patients (28.9%) are being treated outside of CTE/CT, intra-CTE mortality remains high and 6 health zones (Lubero, Masereka, Biena, Manguredjipa, Vuhovi and Mabalako) do not have an ambulance.

ÿ **In Haut-Uélé,** network coverage remains weak in some health zones, and some staff are demotivated.
and Pawa's ambulance is out of service for maintenance.

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2.6. Vaccination

2.6.1. Main actions

ÿ **In Ituri,** vaccination of people living with HIV (PLHIV) using Ervebo and research on the efficacy of the BDBV vaccine continue, bringing the cumulative total to 1,502 people vaccinated (892 men and 610 women, including 21 pregnant women and 35 breastfeeding women) with 29 reported adverse events following immunization (AEFIs) (26 non-severe and 3 severe); the Partners, EBO-PEP, and Oracle trials are ongoing, and the launch of clinical trials for the TUMAINI project is scheduled for October 23, 2026. In Haut-Uélé, vaccination of PLHIV has been suspended. Data from North Kivu and Bas-Uélé are not yet available.

2.6.2. Challenges

ÿ **In Ituri,** the target population for TPL and PPL remains to be redefined and 3 serious AEFI cases have been reported; in Haut-Uélé, PPL vaccination is at a standstill and the breakdown of the generator at the Isiro PEV antenna prevents the 2 CCUs from being operational; in Bas-Uélé, the CCUs are not operational and vaccines and vaccination funds remain insufficient.

2.7. Continuity of Care

2.7.1. Main actions

ÿ **In North Kivu,** partners were sensitized on the CSSE for alignment, the mapping of partners was updated and the analysis of the ESS assessments of the Katwa health zone continues with a view to producing two recovery plans (CH Kyaghala and HGR Katwa).

ÿ **In Haut-Uélé,** the workshop to update the multi-risk contingency plan has concluded with the support of Malteser International, while the workshop to engage non-traditional partners in vaccination (October 23-24) and the community distribution campaign for LLINs are being prepared.

ÿ **In Ituri,** 18 recovered patients (12 in the first follow-up and 6 in the second) received medical and biological follow-up, the rehabilitation of the second clinic for recovered patients continues and the establishment of a clinic for recovered patients in Mongbwalu has been evaluated.

ÿ **In Tshopo,** the evaluation of the Lotoyano polyclinic reveals ineffective triage and 6 out of 12 staff members vaccinated (50.0%).

2.7.2. Challenges

ÿ Recovery plans for CH Kyaghala and HGR Katwa (North Kivu) still to be finalized; poor functionality of triage at HGR Komanda (Ituri); decline in IMCI and deliveries in Tshopo; shortfall in subsidies for free care in the 7 most affected health zones and delay of the catch-up vaccination project (IUI) in Haut-Uélé).

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

In North **Kivu,** 12,817 home visits reached 37,555 people, 414 educational talks reached 7,764 people, 111 mass communications reached 8,244 people, and 3 briefings reached 102 people. 496 community alerts were reported out of 650 expected (76.3%), 31 feedback messages were received out of 45 (68.9%), and 1 out of 8 incidents (12.5%) were resolved within 48 hours. 1,842 people out of 4,750 expected (38.8%) were engaged in community actions. Resistance from young people in Bulongo (Mutwanga) to the construction of a Community Training Center (CTE) was overcome, and media professionals in Katwa and Butembo received training in managing community concerns.

ÿ **In Haut-Uélé,** 587 home visits reached 3,522 people, 12 educational talks reached 592 people and 4 mass awareness campaigns reached 1,292 people; 95 community alerts and 2 death alerts were notified, 15 out of 15 feedback issues were resolved within 48 hours and 2 incidents were recorded (the students ran away from EP Adiba in Pawa and the refusal of EDS in Legu); the training of 30 RECOs began in Isiro.

ÿ **In Tshopo,** 9 mass awareness campaigns reached 10,083 people and 3 educational talks reached 149 people; 2 out of 2 feedback issues were resolved and 2 incidents were recorded (attack on teams at PK 15 and fire at the home of a RECO in Bafwasende).

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ÿ **In Bas-Uélé,** 315 home visits reached 718 people, 2 educational talks reached 41 people and 3 mass awareness campaigns reached 544 people; 6 community alerts (3 living and 3 dead) were notified and the customary chiefs pledged to raise awareness among their populations.

2.9. Mental Health and Psychosocial Support (MHPSS)
2.9.1. Main actions

ÿ **In Ituri,** 33 out of 40 confirmed cases (82.5%) benefited from SMSPS interventions, 93 out of 158 confirmed cases and 135 out of 215 suspected cases are being monitored, and 50 out of 102 new suspected cases have been taken care of. 9 out of 10 households of separated and orphaned children (90.0%) have been supported, 47 children are being monitored, 9 out of 9 children are in daycare, 8 out of 8 have recovered and been reintegrated, 301 caregivers and 86 PPLs have been supported, and 43 results have been announced, including 33 positive results.

ÿ **In North Kivu,** 23 out of 27 confirmed cases (85.2%) benefited from SMSPS interventions, 186 confirmed and 357 suspected cases (122 new and 235 under follow-up) were treated, 37 out of 40 separated and orphaned children households (92.5%) were supported, 28 out of 36 children (77.8%) were followed up in the community, 6 out of 6 children were in daycare, 9 recovered and reintegrated and 93 results announced, including 27 positive; 506 people benefited from psychoeducation sessions and SMSPS focal points in 11 health zones were trained with WHO support.

ÿ **In Haut-Uélé,** 1 new confirmed case, 7 confirmed cases under follow-up and 26 suspected cases (8 new and 18 under follow-up) have been taken care of, 13 results announced, including 2 positive results, 13 separated children followed in the community, 56 accompanying persons and 27 PPLs supported, and 2 recovered patients have left the Isiro CTE.

ÿ **In Tshopo,** all 6 new suspects were taken into custody and 365 people received psychoeducation. In Bas-Uélé, 1 new suspect received support.

2.9.2. Challenges

ÿ **In Ituri,** only 15 of the 28 affected health zones are covered (53.6%) and 18 of the last 37 confirmed cases (48.6%) have not yet been taken care of, notably in Mandima (4/15).

ÿ **In North Kivi,** SMSPS coverage remains weak around the latest confirmed cases in Biena, Manguredjipa, Kayna, Lubero, Masereka, Kalunguta, Mabalako, Mutwanga, and Vuhovi. Ten affected health zones lack childcare facilities, trained personnel, and logistical resources, as well as reintegration kits for recovered patients. In Bas-Uélé, psychosocial support for the three confirmed cases is still lagging. In Haut-Uélé, the mobility of community health workers (Wamba and Boma Mangbetu), computer equipment, and nutritional kits for weaned infants (Pawa and Wamba) are lacking.

2.10. Logistics
2.10.1. Main actions

ÿ **In North Kivu,** IPC, EDS, and MAPI kits were delivered to the health zones of Mutwanga and Beni; the Points of Coordination (POCs) were supplied with infrared thermometers, hand sanitizer, gowns, gloves, and boots; and tarpaulins and barrier nets were delivered to the health zone of Mandima (Ituri); in Tshopo, two additional Toyota Land Cruiser 4x4 vehicles were received, and the Bafwasende sub-coordination received supplies, but 115 motorcycles are still needed in the health zones; in Haut-Uélé, seven three-wheeled vehicles and 25 motorcycles were given to the Bolebole Community Health Workers (CHWs) by the WHO for the village approach; PPE was delivered to the INRB laboratory in Isiro, and work on establishing the Bolebole Treatment Center (TCC) is continuing, but a backup generator is lacking; in Bas-Uélé,
Construction of the CTE at HGR Buta continues (partitioning and toilets), but the diesel fuel stock is exhausted.

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2.11. Security

2.11.1. Main actions

ÿ **In North Kivu,** the burials of confirmed cases at the Bundji and Masiani cemeteries, as well as the decontamination of the Muyisa Hospital (Benengule Health Center), were secured, and a decontamination team narrowly escaped an attack at the Kitaviro Health Center (Musienene). In Tshopo, attacks and threats against teams persist in Bafwasende and at PK 15 (Buta road), where the house of a community health worker was burned down (Kenge Health Center).

ÿ **In Ituri,** insecurity persists in 17 health districts (AS) in Mambasa and Mandima. In North Kivu, the health assessments (EDS) of 4 confirmed cases and the decontamination of 2 health centers (ESS) and one household have been secured. In Tshopo, threats against teams persist in Bafwasende. In Haut-Uélé, reluctance has been reported in Rungu.

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

2.12.1. Main actions

ÿ **In Ituri,** 29 service providers from the Biakato CTE (8 women and 21 men) were briefed on the PSEAH (Protection of Sexual and Harmful Abuse) and signed the code of conduct; 47 people were sensitized at the Kolomani market and during antenatal care visits in Bunia; and 15 survivors of gender-based violence received dignity kits with support from the Red Cross. In North Kivu, 65 service providers in Beni (23 women and 42 men) were briefed on the PSEAH and signed the code of conduct; and awareness sessions on the PSEAH and reporting to 495555/151 reached 150 people in Katwa, 443 in Beni, 42 in Mabalako, 887 in Manguredjipa, 70 in Butembo (32 community health workers and 38 pregnant women), and 155 students in Walikale. In Haut-Uélé, 39 people (9 men and 30 women) were sensitized on the PSEAH and referral mechanisms in Isiro (AS Tely). Nationally, from October 16 to 22, 2026, 2,911 people were sensitized and 364 service providers were briefed in 12 out of 56 health zones (21.4%) and 4 out of 7 health districts, bringing the totals to 73,749 people sensitized and 6,052 signatories of the code of good conduct.

2.12.2. Challenges

ÿ Coverage of affected areas remains low: only 15 out of 36 health zones (41.7%) in Ituri, 6 out of 34 health zones (17.6%) in North Kivu and 6 out of 13 health zones in Haut-Uélé know the reporting mechanism, and 12 out of 56 health zones (21.4%) carried out PSEA activities at the national level this week; only 4 out of 15 partners supported the activities, none of the 14 partners in Haut-Uélé report their PSEA activities, the mapping of stakeholders is not controlled in Ituri, awareness tools are only available in Swahili in Haut-Uélé and CBCMs remain ineffective.

MESSAGES CLES

Over the past 24 hours, 410 out of 451 suspected cases (90.9%) have been investigated in the five provinces that submitted their data; 41 remain uninvestigated (26 in North Kivu, 8 in Wamba, and 7 in Ituri). None of the 23 confirmed suspects in Bas-Uélé have been transferred to a treatment center/evacuee center, and only 2 of the 18 confirmed suspects in Tshopo have been. The contact tracing rate has improved from 84.8% to 89.1%, but remains below the required threshold of 95.0%, with a critical situation in Bambu (28.0%), Lita (50.8%), and Gety (68.1%). One confirmed positive body was intercepted at the Foner Kasindi PoC, and 4 confirmed cases remain lost to follow-up in Bafwasende. This situation requires increased efforts.

the investigation of suspects and the monitoring of contacts in order to limit community risks. of transmission

Maintain a level of vigilance regarding epidemiological surveillance aspects, consolidate actions.

TIMELINE OF KEY EVENTS

o From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

o Between May 17 and 18, 2026, the national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

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